

PROGIT 50 MG TABLETS

Itopride Hydrochloride (50 mg)

DESCRIPTION

White to almost white round biconvex scored film-coated tablets, diameter 7 mm.

The score line is only to facilitate breaking for ease of swallowing and not to divide into equal doses.

COMPOSITION

Each film-coated tablet contains 50 mg of itopride hydrochloride.

Excipients with known effect: Each film-coated tablet contains 74.68 mg of lactose monohydrate.

List of excipients:

Lactose monohydrate, Pregelatinized maize starch, Croscarmellose sodium, Silica colloidal anhydrous, Magnesium stearate and coating composition Opadry II White 85F18422 (partially hydrolyzed polyvinylalcohol, titanium dioxide, macrogol 3350, talc).

PHARMACODYNAMICS

Pharmacotherapeutic group: drugs for functional gastrointestinal disorders, propulsives.

ATC code: A03FA07

Mechanism of action

Itopride activates the gastrointestinal propulsive motility by dopamine D₂ receptors antagonistic action and acetylcholine esterase inhibitory action. Itopride activates acetylcholine release and inhibits its degradation. In addition itopride has an antiemetic action which is based on interaction with dopamine D₂ receptors in chemoreceptor zone.

Itopride accelerates stomach emptying in humans.

Itopride has high specific action in upper part of gastrointestinal tract.

Itopride does not influence plasma concentrations of gastrin.

PHARMACOKINETICS

Absorption

Itopride is absorbed rapidly and almost completely from gastrointestinal tract. Relative bioavailability about 60% is due to liver first pass metabolism. Food does not affect bioavailability of the product. Maximum plasma concentrations are reached in 30 to 45 minutes after administration of 50 mg of itopride.

After repeated administration of doses in the range of 50 to 200 mg 3 times a day for period of 7 days, itopride and its metabolites have shown pharmacokinetics of linear type with minimal accumulation.

Distribution

About 96% of itopride is bound on plasma proteins, mainly albumin. Less than 15% of itopride bound part is bound on alpha-1-acid-glycoprotein.

Biotransformation

Itopride is extensively metabolised in liver in humans. Three metabolites were identified of which only one manifests minor activity without pharmacological significance (about 2 to 3% of itopride effect). The primary metabolite in human is the N-oxide, generated by oxidation of the tertiary amine N-dimethyl group. Itopride is metabolised by flavine monooxygenase (FMO3). The amount and effectivity of human FMO isoenzymes can be associated with genetic polymorphism which can result in rare autosomal recessive condition known as trimethylaminuria (fish odour syndrome). Biological half-life in patients with trimethylaminuria can be longer.

Pharmacokinetic *in vivo* studies of CYP-mediated reactions did not prove inhibition or induction CYP2C19 and CYP2E1 caused by itopride. Administration of itopride did not influence content of CYP or the activity of uridine-diphosphate-glucuronyl transferase.

Elimination

Itopride and its metabolites are primarily excreted by urine. The amount of excreted itopride and N-oxide after oral single therapeutic dose to healthy volunteers was 3.7%, respective 75.4%. Half-life of itopride is about 6 hours.

INDICATION

Itopride is used in the treatment of gastrointestinal symptoms caused by gastric dysmotility and delayed gastric emptying, like sensation of bloating, early satiety, postprandial fullness, upper abdominal pain or discomfort, anorexia, heartburn, nausea and vomiting; functional (non-ulcer) dyspepsia or chronic gastritis.

RECOMMENDED DOSAGE

Posology

The recommended dose for adults is 150 mg daily, [one tablet (50 mg) taken orally three times a day before meals].

The dose may be reduced according to the patient's age and symptoms. The duration of the administration is up to 8 weeks.

Paediatric population

The safety of Progit 50 mg Tablets in paediatric population under the age 16 has not been established.

Elderly

Itopride should be administered in elderly patients with adequate caution because of increased incidence of hepatic and renal function disorders, other diseases or treatment with additional drugs.

Method of administration

Tablets should be swallowed whole with a sufficient amount of liquid.

CONTRAINDICATIONS

Hypersensitivity to the active substance or to any of the excipients.

Progit 50 mg Tablets should not be used in patients in whom increased gastrointestinal motility could be harmful, e.g. in patients with gastrointestinal haemorrhage, mechanical obstruction or perforation.

WARNING AND PRECAUTION

Itopride potentiates acetylcholine action and can induce side anticholinergic effects.

Data about long-term administration of itopride is not available.

This medicinal product contains lactose. Patients with rare hereditary problems of galactose intolerance, total lactase deficiency or glucose-galactose malabsorption should not take this medicine.

INTERACTIONS WITH OTHER MEDICAMENTS

No changes in protein binding have been seen when itopride was administered concomitantly with warfarin, diazepam, diclofenac, ticlopidine, nifedipine and nicardipine.

Drug-drug interactions that arise due to cytochrome P450 metabolism are not assumed because itopride is metabolised mainly by flavine monooxygenase.

Itopride has gastrokinetic effect that could influence the absorption of concomitantly orally administered medicines. Particular attention should be paid to medicines with a narrow therapeutic index, medicines with prolonged-release of the active substance and enteric-coated drug formulations.

Anticholinergic agents may reduce the action of itopride.

Substances as cimetidine, ranitidine, tepronone and cetrexate do not affect prokinetic activity of itopride.

STATEMENT ON USAGE DURING PREGNANCY AND LACTATION

Pregnancy

Itopride should not be used during pregnancy unless the benefits outweigh the potential risks.

Breast-feeding

Data about excretion in mother milk is known only in animals. Because of lack of experience with use of itopride during breast-feeding itopride it is not recommended for breast-feeding women.

ADVERSE EFFECTS / UNDESIRABLE EFFECTS

Adverse reactions have been ranked according to MedDRA terminology under headings of frequency using the following convention:

very common ($\geq 1/10$); common ($\geq 1/100$ to $< 1/10$); uncommon ($\geq 1/1,000$ to $< 1/100$); rare ($\geq 1/10,000$ to $< 1/1,000$); very rare ($< 1/10,000$); not known (cannot be estimated from the available data).

Blood and lymphatic system disorders

Uncommon: Leukopenia.

Not known: Thrombocytopaenia.

Immune system disorders

Not known: Hypersensitivity including Anaphylactoid reaction.

Endocrine disorders

Not known: Hyperprolactinaemia.

Nervous system disorders

Uncommon: Headache, dizziness.

Not known: Tremor.

Gastrointestinal disorders

Common: Abdominal pain, diarrhoea.

Uncommon: Hypersalivation.

Not known: Constipation, nausea.

Hepatobiliary disorders

Not known: Jaundice.

Skin and subcutaneous tissue disorders

Uncommon: Rash.

Not known: Erythema, pruritus.

Reproductive system and breast disorders

Not known: Gynecomastia.

Investigations

Uncommon: ALT increased.

Not known: AST increased, gama-GTP increased, alkaline phosphatase increased, bilirubin increased.

OVERDOSAGE

Overdose was not experienced in humans. In case of overdose the usual measures of gastric lavage and symptomatic therapy should be applied.

STORAGE CONDITION

Store below 30°C.

SHELF-LIFE

36 months.

DOSAGE FORM AND PACKAGING AVAILABLE

PVC/PVdC/Al blister, carton.

Pack size: 20's (1x20), 30's (2x15), 40's (2x20), 90's (6x15) and 100's (5x20) film-coated tablets.

MANUFACTURER:

PRO.MED.CS Praha a.s.

Telčská 377/1, Michle, 140 00, Praha 4, Czech Republic

PRODUCT REGISTRATION HOLDER:

PE Pharma Sdn. Bhd.

15-13A, Wisma UOA 2, Jalan Pinang,

50450 Kuala Lumpur

DISTRIBUTION IN MALAYSIA:

First Pharmaceuticals Sdn Bhd

No. 20 Jalan SS 19/5, 47500 Subang Jaya, Selangor

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